

إِسْمُ الْمَرِيضِ رِبَاعِيًّا : رِضَا أَبُو سَرِيح مَرَسِي حَسَن

المحافظة : الجيزة الحي / المدينة : الهرم

العنوان : 20 ش سلام حسين

تاريخ تسجيل الإستمارة : 22-09-2026

رقم البطاقة : 26905120100484 رقم التلفون : 01117135212

العمر(تاريخ الميلاد) : 12-05-1969 المهنة : حاصل علي دبلوم

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي خط ثاني لورم منتشر عقار (Vinorelbine oral) [14154]

عقار (Vinorelbine oral) [14154]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم حمة العلاج بلائحة المجالس الطبية.

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعمد صحة الشكايات المرفوعة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: رضا ابو سريع مرسى حسن National ID: 26905120100484	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Oral vinorelbine chemotherapy
--

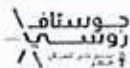
التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



١٤

دms
منزلة الأبحاث
موجا = لوتيه





مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



06/07/2026

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MDT FORM

Name:	Rida Abusreaa Morsy Hassan	MRN:	2762
Age:	57 Y	IO:	26905120100484
Menopausal Sta:	postm	Phone:	01117135212
NRS SCORE:	2	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

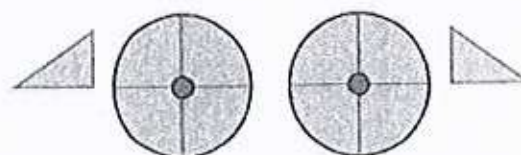
BX: R L
 ER: PR: HER2: KI67:

Sonomammography:**Mets Workup:****History:**

postm left locally advanced left breast cancer met to lung er-ve pr-ve her23+
 rec taxol and 6 herceptin dp rec 3 xeloda 2 tykerb cts 23/2/2026 pd of left
 breast mass pd of left ax in pd of pulmonary nodulespt rec up to c3 d1 gemzar
 carboplatin
 on exam left breast induration left axillary mass fixed mass 6x6 left loq
 ulcerative lesion for mdt bec of clinical progression approval of kadcyle
 pending from ngo

Recommendations:**MDT Outcome & Plan**

for navelbine single agent

**T N M****SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ حنان عزت	د/ احمد لبيب				



CVTS

Name : Reda Abo Serce'a Morsi
Branch : Portable
Ref.Physician Prof : cap+c

Date : 23-02-2026 11:41
Medical No. : GRI-99868
Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

A known case of left breast cancer, as compared to 22/11/2025, the current examination revealed

I- CHEST:

- Progressive course of left breast diffuse heterogenous parenchymal texture and size of the retro-areolar soft tissue mass lesion measures about 7x4 cm compared to 5.84x3 cm, progressive course overlying skin thickness measures about 1.96 cm compared to 1.44 cm
- Progressive course regarding size of ipsilateral enlarged axillary lymph nodes, the largest measures about 4.1x3.28 cm compared to 3.1x3 cm **for further sono-mammography.**
- Rather stationary course of right breast multifocal lesions
- Progressive course regarding size and number of bilateral scattered pulmonary nodules & masses, the largest is seen in left lower lobar superior segment measures about 4.67x3.36 cm compared to 3x2.59 cm
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.

Name : Reda Abo Sereea Morsi

Branch : Portable

Ref.Physician Prof: cap+c

Date : 23-02-2026 11:41

Medical No. : GRI-99868

Ins.Company :

- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- **The examined skeleton** showed no worrisome lytic or sclerotic lesions.

Opinion:

- Progressive course since the last CT examination

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

ECHO REPORT

NAME : رضا أبو سريع مرسى DATE: 9 / 11 / 2025
ID NUMBER : 1799 / ٢٥ SEX : م AGE : ١

EF	60%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVESD	2.5	2-4cm	LA	3.6	2-4cm
FS	29%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GL

SWMA No RWMA

LA and Ao Root Dimensions Normal

Mitral Valve Mild MR

Aortic Valve Mild

Tricuspid Valve Mild TR

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.3

Intracardiac Masses and Effusion

DR.

[Signature]

Patient Name : الأستاذة / رانيا أبو سريع عيسى
Patient No. : 26905120100484
Tab No. : 1961/26
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13009182
Request Date : 31/05/2025
Verification Date : 12/06/2025
Financial Category : فائدة - فائدة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- Ki67: Positive in 40% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, $\geq 1\%$; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $\geq 10\%$ of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within $\geq 10\%$ of the invasive tumor cells or complete and circumferential membrane staining that is intense and within $\leq 10\%$ of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense $> 10\%$ of tumor cells.

Signature:

Dr. Walaa Ebrahim Ghoneim.

Dr. Heba Elwan.

Verified by:

Dr. Duas Mohamed

Printing by : 15/06/2025 10:46:15

Page 2 of 2

Printing by : Mrs. Mayada Hegazy

Patient Name : الأستاذة / رضا أبو سريع مرسى

Patient No. : 26905120100484

Path No. : 1961/25

Sex : Female

Age : 56 Y

Referring Doc. :

Request No. : 13009182

Request Date : 31/05/2025

Verification Date : 12/06/2025

Financial Category : أورام دار السلام - متابعة

Department : Out Patient

Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast 5 O'clock mass.

Nature of Specimen: Tru-cut biopsy, collected at 29/05/2025.

Gross Examination:

- Multiple grey white tissue cores up to 1 cm long. Totally submitted.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

- ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9)

Morphologic Examination:

- Histological type: Invasive duct carcinoma.

- Grade (differentiation: nuclear pleomorphism- mitosis): Grade II (3+3+2).

- DCIS: Not detected.

- Angiolymphatic: Not detected.

- Perineural invasion: Not detected.

- IHC:

- ER: Negative nuclear reaction in tumor cells (score 0/8).

- PR: Weak nuclear reaction in 2% of tumor cells (score 3/8).

- Her2: Strong complete membrane reaction in more than 10 % of tumor cells.

- Ki67: 40% nuclear reaction of tumor cells.

Diagnosis:

- Left breast mass. Tru-cut biopsy:

- Invasive duct carcinoma, Grade II.

- IHC:

- ER: Negative (score 0/8).

- PR: Positive (score 3/8).

- Her2: Positive (score 3+).

Verified by:

Dr. Duaa Mohamed

Printing Date: 13/06/2025 10:46:15

Page 1 of 2

Printing By: Mrs. Mayada Hegazy

MDT FORM

30 / 11 / 2025 G

Name	رحاب سريع	MRN	1799 / 0.00
Age	53y.	ID	079.010.10.012
Menopausal status	postmenopausal	Phone	01177135212
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC T11				R ps:1 L	
ER: -v	PR: +v 3/18	HER 2: 3+	Ki67:		
Sonomammography: post-20 left Breast					
Mets Workup: CT, N2, H1 Lung					
Met CT: Bilat. pulmonary Nodule pr Nec 11/14/25. Tarol + 4 Hepar				Local pD, Left Breast ed, Red 1/1 ↑ sy left axillary LN → Left LOQ men chst for fcyat	
Recommendations:				+ grade 3 pN 11/11/25 (S. H. / S. H.) for MDT after RCT-SC 11/24/25 11/24/25	
MDT OUTCOME & PLAN					

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name: Reda Abo Seree'a Morsi Hassan	Date : 22/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 1699/2025	Pat. ID: 99868

**MULTISCLICE C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS AFTER I.V. CONTRAST INJECTION
REVEALED:**

In reviewing the previous study report dated 20-7-2025, the current study showing:

I- CHEST:

- Left LIQ breas mass measures 5.4 x5.9cm with diffuse parenchymaous edematous changes and overlying skin thickneing.
- Ipsilateral pathologically enlarged axillary lymph nodes,largest measuring about3.4x3.8cm
- Right breat mass wirh fine calcificatio oblong shape with regular border 3.8 x1.5 cm
- Bilateral multiple scattered pulmonary nodules, the largest seen at left lower lobe , the largest measures 2.8 x2.6 cm
- No sizable hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Bilateral small elatstofibroma dorsi,
- No gross cardiomegaly.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Normal C.T appearance of the spleen, both kidneys, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No ascites.

Opinion:

Patient Name: Reda Abo Seree'a Morsi Hassan	Date : 22/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 1699/2025	Pat. ID: 99868

Reported case of left breast cancer showing :

- ❖ Left breast mass with pathological axillary lymph nodes.
- ❖ Right breast mass for sonomamography correlation.
- ❖ Bilateral pulmonary nodules likely metastasis

Dr. Nourhan Mohamed, MD

Signature Dr. Mohamed Hassan
24/11/2025 5:46 PM

Patient's Name: Reda Abu Saree Mursi
Date: 2026-08-23
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH
CONTRAST ADMINISTRATION REVEALED:-**

- An irregular-shaped enhancing density is still seen in the axillary tail of the left breast ,with left axillary lymph node and each measures about 6.1x3.7cm and 4.5x2.5 cm in dimensions respectively. Another enhancing lesion is also seen in the UIQ of the left breast and measures about 14x13mm in dimensions. A third lesion is also seen in the LIQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.
- An enhancing well-defined nodule is seen at 3 O'clock position of the right breast and measures about 15x12mm in dimensions. Another oval shaped solid mass is seen in the LOQ of the right breast with foci of calcifications noted inside and measures about 36x20mm in dimensions.
- Hyperinflated both lungs.
- Increased the size /number of the multiple innumerable subpleural /central enhancing pulmonary solid nodules scattering in both lung fields with the largest one seen in the anterior segment of right upper pulmonary lobe and measures about 91x41 mm in dimensions.
- Linear atelectatic bands are seen in the lingula and in both lower pulmonary lobes with thickened interstitium also noted herein.
- No patchy areas of consolidation or foci calcifications noted.
- Enhancing pretracheal lymph nodes are still seen with no appreciable changes as regards to their size.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed tiny foci of osteosclerosis scattering the scanned lower dorsal and multiple lumbar levels.
- Average sized liver,measuring about 14.6 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.8 cm. No focal lesions seen inside.
- Thickened left adrenal gland is still seen.
- Normal CT appearance of the pancreas, functioning both kidneys, right suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses ,wall thickening or diverticulosis seen.
- A tiny left ovarian cyst is still seen and measures about 24x20mm in dimensions.
- Cuts taken in the rest of the pelvis are unremarkable.

IMPRESSIONS:

**A status follow up and compared with the previous study done at 07/06/2026, the
current study shows: still**

- An irregular-shaped highly-suspicious enhancing neoplastic density (BIRADS-5) still seen in the axillary tail of the left breast ,with left axillary metastatic lymphadenopathy .
- Another enhancing lesion seen in the UIQ of the left breast and a third lesion also seen in

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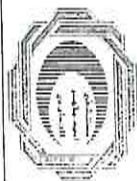
the LUQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.

- An enhancing well-defined nodule still seen at 3 O'clock position of the right breast and another oval shaped solid mass in the LOQ of the right breast with foci of calcifications noted inside. For dynamic MRI of the breasts and /or biopsy.
- Hyperinflated both lungs with multiple variable sized innumerable metastatic subpleural /central enhancing pulmonary solid nodules showing increased size and number,denoting a progressive course,with linear atelectatic bands in the lingula and in both lower pulmonary lobes , thickened interstitium also noted herein , enhancing pretacheal metastatic lymphadenopathy and metastatic thickened left adrenal gland are still seen,denoting a stable course and newly-developed tiny osteosclerotic metastatic bony lesions scattering in the lower dorsal and multiple lumber levels.
- A tiny left ovarian cyst.

Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

Dr.Hanan Alahwel



الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

(Mrs./ Mr. Amr Mohamed Elmaghrabi, 66 years old

female/ male patient presented with diagnosis

MB & to lung

She/ He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She/ He has received (SD IMRT) radiotherapy to

surgeonally Rt breast

At Dose : 20 Gy/ 5 fx/ 1 w .

Additional (Boost) : yes/ no at dose Gy/ fx/ w

(sequential / concomitant)

Started: 2017/2026

ended: 17/7/2026 . With good tolerance .

Head of department
Dr. Hany William

Patient's Name: Reda Abu Saree Mursi
Date: 2026-08-23
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH
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- No ascites.
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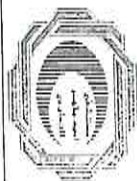
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Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

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مستشفى احمد ماهر التعليمي

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Dr. Hany William

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Referred by: Prof.Dr. no doctor
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- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed tiny foci of osteosclerosis scattering the scanned lower dorsal and multiple lumbar levels.
- Average sized liver,measuring about 14.6 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.8 cm. No focal lesions seen inside.
- Thickened left adrenal gland is still seen.
- Normal CT appearance of the pancreas, functioning both kidneys, right suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses ,wall thickening or diverticulosis seen.
- A tiny left ovarian cyst is still seen and measures about 24x20mm in dimensions.
- Cuts taken in the rest of the pelvis are unremarkable.

IMPRESSIONS:

**A status follow up and compared with the previous study done at 07/06/2026, the
current study shows: still**

- An irregular-shaped highly-suspicious enhancing neoplastic density (BIRADS-5) still seen in the axillary tail of the left breast ,with left axillary metastatic lymphadenopathy .
- Another enhancing lesion seen in the UIQ of the left breast and a third lesion also seen in

Patient's Name:	Reda Abu Saree Mursi
Date:	2026-08-23
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

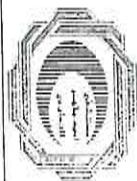
the LUQ of the left breast with pop-corn calcifications noted herein with thickened trabeculations of the left mammary parenchyma and the overlying skin is diffusely thickened as well.

- An enhancing well-defined nodule still seen at 3 O'clock position of the right breast and another oval shaped solid mass in the LOQ of the right breast with foci of calcifications noted inside. For dynamic MRI of the breasts and /or biopsy.
- Hyperinflated both lungs with multiple variable sized innumerable metastatic subpleural /central enhancing pulmonary solid nodules showing increased size and number,denoting a progressive course,with linear atelectatic bands in the lingula and in both lower pulmonary lobes , thickened interstitium also noted herein , enhancing pretacheal metastatic lymphadenopathy and metastatic thickened left adrenal gland are still seen,denoting a stable course and newly-developed tiny osteosclerotic metastatic bony lesions scattering in the lower dorsal and multiple lumber levels.
- A tiny left ovarian cyst.

Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

Dr.Hanan Alahwel



الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

(Mrs./ Mr. Amr Mohamed Elmaghrabi, 66 years old

female/ male patient presented with diagnosis

MB & to lung

She/ He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She/ He has received (SD IMRT) radiotherapy to

surgeonally Rt breast

At Dose : 20 Gy/ 5 fx/ 1 w .

Additional (Boost) : yes/ no at dose Gy/ fx/ w

(sequential / concomitant)

Started: 2017/2026

Ended: 17/7/2026 . With good tolerance .

Head of department

Dr. Hany William

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